



everyday well

What To Take, And What To Skip

CRAVINGS · FOCUS · STEADY ENERGY

*An honest look at the supplements sold for the afternoon crash,
including the ones that don't work.*

Why this is mostly bad news

You asked for this, so the short version first, before you read fourteen pages.

Most of what's sold for cravings, focus and steady energy doesn't do what the label implies. Some of it does. A few things are worth your money, several are worth a try if they're cheap, and a lot of it is expensive urine.

This guide grades every entry one of four ways, and it uses them strictly.

Worth it. Real evidence, in people, at a dose sold in shops.

Worth a try. Plausible, with small or mixed studies behind it, and cheap enough that trying it for a month is a reasonable thing to do.

Thin. Popular, widely sold, and the evidence doesn't support the claim on the front of the bottle.

Skip. Sold hard for this exact problem, and it doesn't do it.

Roughly half of what follows is Thin or Skip. If a guide likes everything, it isn't grading, it's shopping.

This is not a weight-loss list. That is a different promise, and not the one you opted into.



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Before you spend anything

Two things beat every product in this guide, and both are free.

Sleep. One short night raises appetite hormones and lowers the ones that tell you to stop eating, and it does it the following afternoon. If you're running on six hours, your afternoons were always going to be hard, and nothing in a capsule fixes a sleep debt. This is the most common reason the rest of a plan doesn't work.

The three moves from the plan. A five to ten minute walk after a meal, ten slow squats, and thirty grams of protein at breakfast. Working muscle pulls glucose out of your blood without waiting for insulin, so the peak is smaller and the dip that follows is smaller too.

Neither of those is a product, and that is why they go first.

If you have done both for two weeks and the afternoon is still hard, then what you take becomes a fair question. The rest of this is the answer to it.

How to read a label

Three things on a supplement label decide whether you can check it. Learn them and you will be harder to sell to, including by us.

A proprietary blend is a number you can't check. When a label says "Metabolic Blend 1,200 mg" and then lists eight ingredients underneath, the only thing you know is that all eight together weigh 1,200 mg. The first one might be 1,100 mg and the last one a pinch. Blends aren't automatically dishonest and a company can have a real reason for one. But if you can't itemize it, you can't hold it against a study. **An ingredient with a named amount beside it can be checked. Anything inside a blend cannot.**

"Clinically studied ingredient" isn't "clinically studied product." The first says somebody once ran a trial on a substance that appears somewhere in this bottle, at a dose and a form that may have nothing to do with what's in your hand. The second says they tested this. The second is rare and companies say so loudly when it's true.

The dose is where the claim usually dies. Most ingredients that work do so within a range, and a lot of products carry the right ingredient at a fraction of the studied amount. Where this guide knows the studied dose, it's printed, so you can hold a label up against it.

Two smaller tells. Look for a third party seal (NSF, Informed Choice, USP): it does not prove the product works, it proves the bottle contains what the label says. And look at where the amount sits. A named amount on the front and a blend on the back usually means the named one is the only one at a real dose.



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Cravings, the ones with something behind them

Protein and fiber.

WORTH IT

Not a supplement, and the most effective thing on this list. Thirty grams of protein at breakfast measurably reduces snacking later in the day, and fiber slows the climb after a meal. A protein powder counts if it's easier than cooking.

Glucomannan.

WORTH A TRY

A soluble fiber that absorbs water and takes up room. The evidence is modest and mostly sits alongside an energy-restricted diet, but it's cheap and the mechanism isn't mysterious. Around 1 g before meals, three times a day, **with a full glass of water**. Do not take it dry or straight before lying down: it swells, and the choking and obstruction risk is real.

Saffron extract.

WORTH A TRY

Small trials suggest it reduces snacking between meals, usually around 88 to 176 mg a day of a standardized extract. The studies are small and several were funded by the people selling the extract, which is worth knowing before you decide the case is settled.

Green tea extract (EGCG).

WORTH A TRY

with a real caution

A small effect on metabolic rate and fat oxidation, usually studied alongside caffeine, and much smaller than the marketing suggests. ⚠️ **Concentrated green tea extract has been linked to liver injury**, with the concern focused on high doses taken on an empty stomach. European authorities flagged doses at and above 800 mg EGCG a day from supplements. Drinking green tea isn't the issue. If you take an extract, take it with food, stay well under that, and stop if you feel unwell.

Cravings, the ones that don't hold up

Chromium. THIN

Sold everywhere for carb cravings and blood sugar. The trials are inconsistent, the effects are small where they exist at all, and the clearest signal is in people who are low in chromium to begin with, which is uncommon. Typical doses are 200 to 1,000 mcg. It's safe and it's cheap, which is the only argument for it.

Cinnamon. THIN

A few meta-analyses show a small effect on fasting glucose and plenty of others show nothing, with the studies disagreeing on dose, form and who it helps. One thing worth knowing if you use it anyway: **cassia cinnamon, the ordinary supermarket kind, contains coumarin**, which is hard on the liver in quantity. Ceylon cinnamon has very little. A supplement using Ceylon has made the better choice.

Berberine. WORTH A TRY *and read the cautions twice*

This one is the opposite of the two above: the glucose-lowering evidence here is decent, which is rare in this guide, typically 500 mg two or three times a day. But it upsets a lot of stomachs, **it interferes with the enzymes that clear many prescription drugs**, which can raise their levels in your blood, and it isn't for anyone pregnant or breastfeeding. If you take any prescription medication, this is a conversation with a pharmacist, not an impulse buy.

Apple cider vinegar. SPLIT VERDICT

The **liquid** is a fair *worth a try*: a tablespoon in water before a carb-heavy meal blunts the rise afterwards a little, and the acetic acid is what does it. The **gummies** are a *Skip*. The acid is the active part and it's the part a gummy has least of, along with the sugar a gummy needs to taste like a sweet.

Garcinia cambogia. SKIP

Heavily marketed for appetite, repeatedly tested, and the better-run trials find nothing beyond placebo.

Raspberry ketones. SKIP

The work behind the claim is in cells and rodents at doses no person could take. There are no useful human trials.

Detox teas. SKIP

Most work by being a laxative, usually senna. You lose water, and you can end up dependent on a laxative.



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Focus

Caffeine. **WORTH IT** *and be honest about what it does*

It's the best-evidenced thing in this guide for alertness and reaction time. What it doesn't do is remove an afternoon crash. It postpones one and creates a smaller one later, which is why the four o'clock cup is a rescue dose rather than a plan. Useful range is roughly 100 to 200 mg, about one to two cups of coffee. **Half of it is still in you five hours later**, and in the trials people who drank caffeine six hours before bed lost close to an hour of sleep a night without noticing. The FDA's general ceiling for healthy adults is 400 mg a day from everything combined.

L-theanine with caffeine. **WORTH IT**

An amino acid from tea leaves that takes the edge off caffeine without taking the alertness with it, and the pairing is better studied than either alone for calm focus. Around 100 to 200 mg of theanine alongside your caffeine. It isn't exciting and it holds up better than most of this page.

L-tyrosine. **WORTH A TRY**

A building block for the neurotransmitters behind attention and drive. The evidence is decent where the brain is under load, meaning stress, cold, or a night without sleep, and much weaker for an ordinary Tuesday. Studied around 100 to 150 mg per kilogram of body weight, which is a far bigger dose than most products contain.

Rhodiola rosea. **WORTH A TRY**

Consistently studied for mental fatigue, usually 200 to 400 mg a day of an extract standardized to 3% rosavins. The trials are small and short, and the effect, where it shows up, is on feeling less worn down. Nobody thinks faster on it.

A note on the word "nootropic." It has no legal or regulatory meaning. It isn't a category a product has to qualify for. Read what's in the bottle and at what dose, and ignore the word entirely.



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Steady energy, and why this page is short

This is the page with the least good news on it, because "energy" in a supplement almost always means "stimulant" or "vitamin you already have enough of."

B vitamins. THIN *unless you're short of them*

B12, B6, folate and niacin are required to turn food into energy, and that's where the marketing comes from. It doesn't follow that more helps when you already have enough, and for most people eating a mixed diet, you do. **The real exceptions are worth knowing:** people who eat no animal products, people over sixty, people on metformin or long-term acid reducers, and anyone pregnant or trying. If you're in one of those groups, B12 and folate are a genuine *Worth it*.

Iron. WORTH IT *if you're low, and a bad idea if you aren't*

Low iron is a common and very treatable cause of being tired all afternoon, especially in women who menstruate. It's also the one entry in this guide you should not buy on a hunch: excess iron accumulates and is harmful, so this is a blood test first and a supplement second.

Magnesium. WORTH A TRY

Modest, cheap, and a lot of people run low. The form matters more than the brand: glycinate and citrate absorb well, oxide mostly doesn't and is what you find in the cheapest tubs. 200 to 400 mg in the evening. Expect a small improvement in sleep quality. Your afternoon will feel the same.

Vitamin D. WORTH IT *if you're low*

Tiredness is one of the ways deficiency shows up and it's common in winter and indoors. Same rule as iron: test rather than guess, because the dose depends on how low you are.

Most energy supplements sell the feeling of having done something about being tired. What moves it is sleep, iron if you are low, and not crashing at three o'clock.

Where a blend fits, and the one I take

Everything so far has been single ingredients, because a single ingredient is easy to judge. It's also a narrow thing to judge.

Appetite, mood, energy and how your body handles fuel are separate systems. A single lever reaches one of them. That's the argument for a formula, and a formula earns it by saying what's in it and how much.

So here is the one I take, graded like everything else in this guide.

Nueva Beyond Daily. **WORTH A TRY** *and you can check four of its twelve amounts*

⚠ Disclosure, and it belongs on this page rather than in the small print at the back: the person who sent you this guide sells this product. That's the whole reason you can read it for free. Everything above is what I would have told you anyway, and this entry is the one where you should be hardest on me.

What it contains, from the published material: a 1,178 mg blend called BeyondMI Daily Edge holding L-tyrosine, Ceylon cinnamon extract, rhodiola rosea, theobromine, a grape and apple polyphenol extract, and saffron extract. Beside the blend, with named amounts: **caffeine 150 mg, chromium 300 mcg, niacin 15 mg NE, vitamin B6 5 mg, folate 680 mcg DFE, vitamin B12 50 mcg.**

How that scores against the last five pages. Caffeine at 150 mg is a real dose, inside the useful range, and it's the ingredient doing the most obvious work. Saffron, rhodiola and tyrosine are all *worth a try* entries in their own right. Ceylon rather than cassia cinnamon is the better of the two choices, though cinnamon is *Thin* either way, and so is chromium, at any dose. The B vitamins matter if you're in one of the groups on the last page and do very little if you aren't.

What I can't tell you, and neither can the label. The blend isn't itemized, so there is no way to know whether the saffron inside it is at the 88 to 176 mg that was studied or at a tenth of it. That applies to the rhodiola and the tyrosine too. Four of the twelve amounts are published and eight aren't. A supplement facts panel would settle it and I haven't seen one.

One practical thing. 150 mg of caffeine is roughly a strong cup and a half, and half of it is still in you five hours later. Take it in the morning. Taking a caffeinated capsule at two o'clock to fix an afternoon crash is the rescue-dose loop from the first guide, in a more expensive format.

Nueva's own framing, in their words: the intent was to build the product around several areas at once "instead of building the Beyond story around one ingredient, one pathway, or one promise." Whether that's worth it to you depends on how much you value one capsule over four.

If you take one thing from this

Do these in order.

1. Sleep, for two weeks, properly. If your afternoons improve you have your answer, and you have spent nothing.

2. The walk after lunch. Five to ten minutes. This is the highest-return thing in either guide and it also costs nothing.

3. Then, if you still want to, one product at a time for a month. Not a stack. One thing, so that when something changes you know what changed it.

How to run that test on yourself. Before you start, write down two numbers: how many days last week you wanted something sweet at 3pm, and how many days you got through the afternoon without a rescue coffee. Take the thing for four weeks. Write the same two numbers again. It's a crude experiment, and it's more than most people do.

What would make you stop. No change in the two numbers after a month. Any new symptom. Anything your doctor or pharmacist raises an eyebrow at. None of this is worth defending.



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Message whoever sent you this

You got this guide from a person, not from a company, and they can do two things a PDF can't.

They can answer the one question you came with. Most people don't need the whole list. They need to know whether the one thing they were about to buy is any good, and whether it matters that they take a medication.

They can tell you what it costs. Including whatever is running at the moment, which this document can't know.

Ask them anything in here. Ask them what they take and whether it did anything. Ask them what they would skip.

If the honest answer to your question is "nothing in a bottle will fix that," a good one will tell you so.

The fine print

This guide is general information about dietary supplements. It is not medical advice, it is not a diagnosis, and it is not a treatment plan. Nothing in it is intended to diagnose, treat, cure or prevent any disease. These statements have not been evaluated by the Food and Drug Administration.

Dietary supplements are not reviewed for safety or effectiveness before they are sold. Talk to your doctor or pharmacist before starting anything here, and especially if you take prescription medication, have a medical condition, are pregnant or breastfeeding, or are giving anything to a child.

Specific cautions repeated from the pages above, because they are the ones that matter: concentrated green tea extract has been associated with liver injury and should not be taken in large doses on an empty stomach. Berberine interferes with the enzymes that clear many prescription drugs. Glucomannan must be taken with plenty of water and never immediately before lying down. Iron should be taken on the basis of a blood test. Keep total daily caffeine from all sources, including coffee, tea and any supplement, within the 400 mg the FDA describes as generally safe for healthy adults, and less if you are pregnant or sensitive to it.

Doses given here are the ones used in published studies. They are printed so you can compare a label against them, not as a recommendation for you.

Individual results vary, and most of the studies referenced are small.

The person who sent you this guide sells one of the products in it, and says so on the page where it appears.